THE WHOLE TREATMENT JOURNEY

Dose-dense AC → weekly paclitaxel: the whole pathway

Neoadjuvant or adjuvant · early breast cancer

Four AC treatments 14 days apart, followed 14 days after the last AC by 12 weekly paclitaxel doses, subject to recovery and review.

Read from left to right. Columns show stages, not equal lengths of time. This is a general plan: no dates or personal information are needed. Your team confirms which steps apply and arranges appointments.

Treatment, tests and reviews across the course

Care / check	Preparation	AC · about 8 weeks	Paclitaxel · 12 weeks	Local treatment	Continuing care
Cancer medicines NCCP 00485	Confirm indication and fertility plan	AC every 14 days ×4; G-CSF each cycle	Paclitaxel weekly ×12	Recovery; MDT confirms next phase	Endocrine or HER2 treatment only if separately indicated
Surgery EARLY	Agree lumpectomy / mastectomy and nodes	If adjuvant, surgery already completed	If neoadjuvant, surgery follows chemotherapy	Recovery then RT if indicated	Rehabilitation and late-effect review
Radiotherapy EARLY	Assess likely need	Usually not concurrent with AC	Usually after chemotherapy	After surgery and recovery if indicated	May overlap later endocrine treatment
Blood tests NCCP 00485	FBC, kidney/liver tests	Before each AC treatment	Before each weekly dose	Confirm recovery for surgery / RT	Tests according to ongoing medicines
Heart checks NCCP 00485 · ESC	Baseline ECG; echo / LVEF assessment	Doxorubicin exposure and risk review	Symptoms or risk determine extra checks	Communicate prior anthracycline exposure	Risk-based post-treatment review
Cancer imaging EARLY · BONE	Breast imaging; staging if indicated	Clinical response if before surgery	Image for surgery planning when indicated	Operation and pathology if neoadjuvant	Annual mammography of remaining breast tissue

What each check is for

1

Blood tests and treatment review

FBC, kidney/liver tests → Before each AC treatment → Before each weekly dose → Confirm recovery for surgery / RT → Tests according to ongoing medicines

Results may change whether a dose is given or held.

Ask / act: Confirm the booking and who reviews the result before treatment.

Sources: NCCP 00485

2

Heart tests

Baseline ECG; echo / LVEF assessment → Doxorubicin exposure and risk review → Symptoms or risk determine extra checks → Communicate prior anthracycline exposure → Risk-based post-treatment review

ECG measures rhythm; echo measures pumping function. They are different checks.

Ask / act: Keep the planned appointments and report symptoms between them.

Sources: NCCP 00485 · ESC

3

Imaging and markers

Breast imaging; staging if indicated → Clinical response if before surgery → Image for surgery planning when indicated → Operation and pathology if neoadjuvant → Annual mammography of remaining breast tissue

The purpose and frequency depend on early versus advanced disease.

Ask / act: Ask which test is next and what decision its result will inform.

Sources: EARLY · BONE

4

Bone health with endocrine treatment

At endocrine / ovarian-suppression planning; repeat by risk

Low oestrogen can affect bone strength.

Ask / act: Ask whether DEXA, fracture-risk assessment or bone protection is appropriate.

Sources: BONE · NOGG

5

Who arranges and reviews each check?

At initiation and every handover

A recommended test can be missed if responsibility is unclear.

Ask / act: Ask the team to document the booking owner, result reviewer, due window and escalation route.

Sources: NCCP 00485

At the next review, ask

- Which treatment phase am I in, and what happens next?
- Which test is expected next, who arranges it, and who explains the result?
- Who should I contact if an expected appointment has not arrived?

This map does not show whether a test has been ordered, booked or completed. Do not wait for a scheduled test if you develop new symptoms.